

Content Guide to Behavioral Health Care

- Behavioral Health Care Description
- Behavioral Health Care Overview
- Database Analysis - Behavioral Health Care
- Data Resources
- Behavioral Health Care Title and Code
- Behavioral Health Level of Care Guidelines
 - Admission Guidelines
 - Recovery Course - Behavioral Health Level of Care Guidelines
 - Social Determinants of Health Assessment
 - Care Planning and Evaluation
 - Discharge Guidelines
 - Discharge Planning
- Care Guidelines for Behavioral Health
 - Pediatric Care Guidelines for Behavioral Health
 - Care Planning - Admission and Alternatives
 - Clinical Indications for Admission
 - Alternatives to Admission
 - Alternative Care Planning
 - Hospitalization and Treatment
 - Recovery Course - Care Guidelines for Behavioral Health
 - Social Determinants of Health Assessment
 - Goal Length of Stay and Extended Stay (Inpatient Only)
 - Care Planning
 - Discharge Criteria
 - Discharge
 - Discharge Planning
 - Discharge Destination
 - Evidence Summary
 - References
 - Footnotes
- Other Behavioral Health Care Guidelines
 - Clinical Indications
 - Current Role Remains Uncertain
 - Alternative Care Planning - Other Behavioral Health Care Guidelines
 - Care Treatment
 - Evidence Summary - Other Behavioral Health Care Guidelines
 - References
 - Footnotes
 - Definitions
- Behavioral Health Scoring Tools
- Home Care and Recovery Facility Behavioral Health Guidelines
 - Home Care Visit Data
 - Home Care Visit Data Methodology
 - Recovery Facility Care Length of Stay Methodology
- Abbreviations
- Care Management Tools

Behavioral Health Care Description

Behavioral Health Care provides evidence-based guidelines to help healthcare professionals guide the effective treatment of patients with psychiatric disorders. There are 7 main guideline types in Behavioral Health Care: Behavioral Health Level of Care Guidelines, diagnosis-specific Care Guidelines for Behavioral Health, Opioid Management, Specialty Medications, Community Services, Testing Procedures, and Therapeutic Services.

Behavioral Health Care Overview

The purpose of the care guidelines is to promote evidence-based care and identify benchmark patient care and recovery milestones to enhance the delivery of quality healthcare and promote more efficient resource management across the continuum of care. Consistent with that continuum-of-care approach, Behavioral Health Care provides guidelines for the treatment of psychiatric disorders ranging from major depressive disorder and anxiety disorders to schizophrenia, as well as eating disorders and substance-related disorders. The appropriateness of specific psychological, behavioral, and pharmacologic therapies is addressed, and indications are presented for 5 different levels of care (inpatient care, residential care, partial hospital program, intensive outpatient program, and outpatient care) to help define the optimal level of care for effective, efficient behavioral health therapy. The guidelines can assist case managers and other mental healthcare professionals in developing outpatient alternatives to higher levels of care, tracking patient progress during treatment within a level of care, facilitating the progress of patients whose recovery is delayed, and preparing comprehensive plans for transition of patients from one level of care to another.

Database Analysis - Behavioral Health Care

In addition to the published literature, the guidelines have been developed and validated by using the reported utilization experience from various databases. These databases include nationally representative samples of inpatients and outpatients. In terms of guideline development, the purpose of database analysis is to confirm the reasonability and clinical appropriateness of care guideline utilization goals and objectives.

This process supplements the review of the published literature. Published studies, even when they address the same general topic, are only occasionally designed to elucidate the specific resource management issues addressed in the guidelines. In addition, they are often based on subject recruitment criteria that are not representative of the entire population of patients a guideline covers. For those reasons, database analysis provides a useful additional perspective.

One of the principal uses of database information is to verify the reasonability of the inpatient level of care Goal Length of Stay (GLOS). When cited in a guideline, the database analysis is reported as a percentage of patients in the United States who are discharged from an inpatient admission at or before the GLOS. Typically, the text reads: "Analysis of national hospital discharge data shows..." The source citation is: Premier PINC AI™ Healthcare Database (PHD), 01/01/2021-12/31/2022. In cases where sufficient data does not exist, 4 years of data are used: Premier PINC AI™ Healthcare Database (PHD), 01/01/2019-12/31/2022. This database is not restricted by payer (eg, Commercial, Medicare, Medicaid represented), and includes patients of all ages (except where noted, for example in Child or Adolescent guidelines). All deaths and transfers to or from another acute facility are excluded.

All inpatient-care discharges of patients with all payers are used in the length of stay analysis. These discharges include patients of all ages (except where noted) covered under group or individual insurance plans, Medicare, Medicaid, or other non-insurance-based payments. All deaths and transfers to or from another acute facility are excluded.

Data Resources

Behavioral Health Care provides several statistical benchmarking tools, including Behavioral Health Inpatient GLOS Attainment Analysis, Behavioral Health Care Level of Care Statistics, and Behavioral Health Utilization Models.

Behavioral Health Care Title and Code

For identification purposes, each guideline in Behavioral Health Care is assigned a Guideline Code, for example, B-008-PHP (Major Depressive Disorder: Partial Hospital Program). The prefix "B" designates Behavioral Health Care, and the suffix PHP corresponds to the level of care, in this case, partial hospital program.

Behavioral Health Level of Care Guidelines

The Behavioral Health Level of Care Guidelines are organized predominantly according to levels of care rather than specific diagnostic groups. The 5 main levels of care are: inpatient care, residential care, partial hospital program, intensive outpatient program, and outpatient care. In addition to these 5 main levels of care, there are 5 levels of care provided to address special circumstances and include day treatment, long-term community-based residential care, custodial care, crisis intervention, and observation care. For information on behavioral health levels of care, see Behavioral Health Levels of Care [SR](#).

"Adult" and "Child or Adolescent" guideline versions are available for inpatient, residential, partial hospital, intensive outpatient, and outpatient levels of care, for a total of 10 general Behavioral Health Level of Care Guidelines. There are separate guidelines covering each of these levels of care for eating disorders (5 levels each for adult and child/adolescent, for a total of 10 guidelines) and substance-related disorders (5 levels each for adult and child/adolescent, for a total of 10 guidelines). These guidelines are intended for use in initial assessment and determination of the patient's level-of-care needs. A general recovery course with criteria for continued stay and discharge allows ongoing management of the case.

Level of care comparison charts are available for side-by-side comparison to aid in level-of-care decisions. There are 6 level of care comparison charts, including Adult and Child or Adolescent versions of general behavioral health levels of care, eating disorders, and substance-related disorders.

Use of the Level of Care Guidelines vs the disorder-specific Care Guidelines for Behavioral Health (which are addressed separately in this Content Guide) is at the user's discretion. An example of a potential use-case for the Level of Care Guidelines, rather than the disorder-specific Care Guidelines for Behavioral Health, is the ability to facilitate delivery of care at the appropriate level of service intensity when a specific diagnosis has not been determined. The user is encouraged to initially use or change over to the use of a diagnosis-specific Care Guidelines for Behavioral Health guideline whenever appropriate in the patient's course of care, as the diagnosis-specific guidelines contain more detailed and elaborate content relevant to the given diagnosis.

Admission Guidelines

This section lists specific criteria that indicate the need for a patient to be admitted to the level of care. If none of the criteria are present in the index case, the need for admission to the level of care may be questionable. With the complexity of behavioral health care issues, however, there may well be other reasons for admission. As always, the care guidelines are designed to assist the clinical review process that is needed to render judgment about cases in which admission may be appropriate. Admission to a specified level of care may be necessary even when the indications listed for that level of care are not present.

There are separate Adult and Child or Adolescent admission criteria for the 5 main behavioral health levels of care (inpatient, residential, partial hospital, intensive outpatient, and outpatient). In addition, because of the relatively specific nature of the criteria, there are separate Adult and Child or Adolescent admission criteria for each of the 5 main levels for eating disorders and substance-related disorders. The admission criteria listed in the 5 main Behavioral Health Level of Care Guidelines are identical to those in the Clinical Indications for Admission section in the diagnosis-specific Care Guidelines for Behavioral Health for the same level of care.

In addition to the 5 main levels of care, Admission Guidelines are provided for 5 special situations: day treatment, long-term community-based residential, custodial care, crisis intervention, and observation.

Recovery Course - Behavioral Health Level of Care Guidelines

The Recovery Course contains 2 stages. Recovery Milestones are used to determine if ongoing care, interventions, and documentation indicate that continued care at the level is warranted. Absence of achievement of milestones should prompt questions about the expected timing of unmet milestones and appropriateness of continuing the level of care. Meeting all Recovery Milestones indicates continued care is appropriate until Stage 2 Recovery Milestones are met.

Social Determinants of Health Assessment

Patients at higher risk for an unmet health-related social need can be identified using this assessment upon admission and throughout the patient's stay. There is a growing body of evidence that unmet health-related social needs can have a negative impact on quality of life and health outcomes, and results from the social determinants of health assessment should inform individual treatment plans and identify potential interventions to facilitate discharge planning and transitions of care. The assessment is available in the Behavioral Health Care Level of Care Guidelines and in the Care Guidelines for Behavioral Health (as well as in all inpatient guidelines in Inpatient & Surgical Care) and may be answered by the patient, a parent, or a caregiver. The assessment covers housing insecurity, food insecurity, insufficient transportation, insufficient utilities, personal safety risk, insufficient dependent care, and depression risk. The assessment can be accessed from a pop-up bullet in the Clinical Status column in the Recovery Course table.

Care Planning and Evaluation

The Care Planning and Evaluation section contains information on common therapy, discharge planning needs, and other care items that might be expected during the patient stay in the defined level of care. This section helps care planners assess possible care needs and appropriateness of care being rendered.

Discharge Guidelines

The Discharge Guidelines section reproduces the defining criteria for each of the Recovery Milestone bullets in stage 2 of the Recovery Course. The criteria identify patients who may be appropriate for discharge from the specific level of care due to adequate patient stabilization or improvement. In addition, the criteria identify patients for whom the level of care is no longer indicated because a higher level of care is indicated or because the patient or guardian refuses treatment.

Discharge Planning

The Discharge Planning section summarizes the discharge planning needs that are most important to accomplishing a safe discharge and immediate postdischarge course of care. It covers tasks that may need to be accomplished during the patient's stay or at the time of discharge from the relevant level of care.

Care Guidelines for Behavioral Health

The Care Guidelines for Behavioral Health are organized into 16 diagnostic groups, each of which comprises a single diagnosis or cluster of similar diagnoses (2 of these, "Other Psychiatric Disorders" and "Other Psychotic Disorders," can be used for less common diagnoses that are not assignable to any of the other 14 groups).

For 13 of the 16 diagnostic groups (excluding Dementia, Delirium, and Substance-Related Disorders), there are a total of 6 level of care guidelines: (1) inpatient adult, (2) inpatient child or adolescent, (3) residential (including both adult and child/adolescent admission criteria), (4) partial hospital (including both adult and child/adolescent admission criteria), (5) intensive outpatient (including both adult and child/adolescent admission criteria), and (6) outpatient (including both adult and child/adolescent admission criteria). The inpatient adult and inpatient child or adolescent guidelines also include age-specific Patient Education for Clinicians and Discharge Information.

For Dementia, there are 5 adult level of care guidelines (inpatient, residential, partial hospital, intensive outpatient, and outpatient). In addition, Patient Education for Clinicians and Discharge Information are provided.

For Delirium, 1 inpatient guideline is provided, along with Patient Education for Clinicians and Discharge Information.

For Substance-Related Disorders, there are 2 complementary sets of level of care guidelines. For comprehensive combined substance-related disorders treatment and withdrawal management, there are 6 Substance-Related Disorders level of care guidelines: (1) inpatient adult, (2) inpatient child or adolescent, (3) residential (including both adult and child/adolescent admission criteria), (4) partial hospital (including both adult and child/adolescent admission criteria), (5) intensive outpatient (including both adult and child/adolescent admission criteria), and (6) outpatient (including both adult and child/adolescent admission criteria). The inpatient adult and inpatient child or adolescent guidelines also include age-specific Patient Education for Clinicians and Discharge Information. For care that is limited to treatment of withdrawal, there are 5 Withdrawal Management level of care guidelines: (1) inpatient adult, (2) residential adult, (3) partial hospital adult, (4) intensive outpatient adult, and (5) outpatient adult. The inpatient adult guidelines also include Patient Education for Clinicians and Discharge Information.

Pediatric Care Guidelines for Behavioral Health

For 14 of the 16 diagnostic groups (excluding Dementia and Delirium), the inpatient optimal recovery guidelines include pediatric guidelines that focus on behavioral health conditions in children and adolescents; Withdrawal Management within the Substance-Related Disorders diagnostic group does not include pediatric guidelines. The Child or Adolescent guidelines are intended to apply to patients younger than 18 years of age. They contain pediatric-specific citations and content, such as pediatric-specific Goal Length of Stay (GLOS) support. They also include age-specific Patient Education for Clinicians and Discharge Information.

Care Planning - Admission and Alternatives

This section is used to identify the patient who is appropriate for admission and to optimize the location of care.

Clinical Indications for Admission

The Clinical Indications for Admission section lists specific criteria that indicate the need for a patient to be admitted to a specific level of care. If none of the criteria are present in the index case, the need for admission may be questionable. With the complexity of behavioral health care, however, there may well be other reasons for admission. As always, the care guidelines are designed to assist the clinical review process that is needed to render judgment about cases in which admission may be appropriate. Admission to that level of care may be necessary even when the indications listed for that level of care are not present. The guidelines are not inflexible expectations for all patients but should be considered as guides to quality care. The care guidelines are not designed to be used in isolation but rather in conjunction with clinical judgment.

Alternatives to Admission

The Alternatives to Admission section lists the other levels of care that might be used to care for the patient, and it provides links to guidelines on those alternative levels of care.

Alternative Care Planning

The Alternative Care Planning section lists care steps that may need to be performed or arranged while pursuing another level at which to provide care for the patient. This section is designed to help providers and care managers anticipate the common tests, procedures, and other care steps that may need to be planned in the other care levels. Generally, only some of the items listed will apply for any given patient.

Hospitalization and Treatment

The Hospitalization section of inpatient Care Guidelines for Behavioral Health includes an Optimal Recovery Course, a Goal Length of Stay, an Extended Stay section, and information on care planning and discharge. The Treatment section of non-inpatient Care Guidelines for Behavioral Health includes a Recovery Course, information on care planning, and discharge criteria.

Recovery Course - Care Guidelines for Behavioral Health

The care guidelines for inpatient treatment describe hospitalization in terms of the reasonable best-case scenario for the diagnosis covered by the guideline. Inpatient guidelines describe an Optimal Recovery Course in which the patient progresses as well as can be

expected. This patient receives care that is marked by optimal decision making, optimal physician and other healthcare provider performance, and optimal delivery system capabilities. Deviations from the Optimal Recovery Course may occur for a variety of reasons, including primary patient factors (eg, nonoptimal recovery), secondary patient factors (eg, complications due to drug reactions, which may slow recovery), and provider or system factors (eg, therapy available each day it is needed).

The Optimal Recovery Course describes the key care steps and milestones in a patient's recovery. It incorporates information gleaned from searching the published literature, including professional society guidelines. The patient's status and care elements are presented in 5 categories: Clinical Status (eg, no dangerous behavior), Interventions (eg, possible behavioral crisis management), Medications (eg, possible cholinesterase inhibitor or memantine), Evaluation (eg, presenting symptoms assessed), and Therapy (eg, psychosocial interventions). Key care steps are listed for each day of the pathway that they are to be provided, and changes in care, discontinuation of important care items, and changes in status are noted. Absence of content for a particular category on a particular day merely means there are no specific key steps identified for that diagnosis and that day. Footnotes provide additional information regarding care protocols and patient status criteria. If a bulleted item appears on more than one day, the footnote is listed for the first day the item appears; the identical information may not be repeated on subsequent days.

Status and care steps that represent significant change and indicate the patient has progressed to a particular day are identified as Recovery Milestones and are presented in bold. The final day of the pathway lists Recovery Milestones (some of which may be repeated from other days) that identify that the patient may be ready for discharge to a lower level of care.

Guidelines for non-inpatient hospital levels of care include a Recovery Course table that shows progress in stages of flexible duration. Recovery Milestones (shown in bold) for the last stage correspond to a set of Discharge Criteria, which indicate when it may be appropriate to discharge the patient to a lower level of care.

Social Determinants of Health Assessment

Patients at higher risk for an unmet health-related social need can be identified using this assessment upon admission and throughout the patient's stay. There is a growing body of evidence that unmet health-related social needs can have a negative impact on quality of life and health outcomes, and results from the social determinants of health assessment should inform individual treatment plans and identify potential interventions to facilitate discharge planning and transitions of care. The assessment is available in the Behavioral Health Care Level of Care Guidelines and in the Care Guidelines for Behavioral Health (as well as in all inpatient guidelines in Inpatient & Surgical Care) and may be answered by the patient, a parent, or a caregiver. The assessment covers housing insecurity, food insecurity, insufficient transportation, insufficient utilities, personal safety risk, insufficient dependent care, and depression risk. The assessment can be accessed from a pop-up bullet in the Clinical Status column in the Recovery Course table.

Goal Length of Stay and Extended Stay (Inpatient Only)

The Goal Length of Stay (GLOS) identified in inpatient guidelines is the length of stay that could be achieved in a substantial proportion of patients with optimal recovery, decision making, and health system design and performance. Patients should be discharged to a lower level of care when it is appropriate for their clinical status and care needs regardless of whether that occurs on, before, or after the goal. If alternative levels of care are not available, patients may require continued hospitalization. Using the GLOS in isolation, out of context with the guidelines, is inappropriate. Only proper care and sufficient healthcare system resources allow the GLOS to be attained in a high-quality cost-effective manner.

It is expected that not all patients will be discharged by the GLOS. The GLOS in Behavioral Health Care is a benchmark to be used as a target. It is not the mean, median, or modal length of stay. The frequency with which the GLOS can be achieved varies among guidelines because such factors as comorbidities, severity of mental illness, and implementation of new practices may vary from diagnosis to diagnosis.

The Extended Stay section in the inpatient care guidelines describes situations in which the hospital stay will often be longer than the GLOS. Brief descriptions of the expected evaluation or therapeutic regimen for these conditions, combined with estimates of extended stay duration, are included. The general period of time for these estimates is: Minimal (a few hours to 1 day), Brief (1 to 3 days), Moderate (4 to 7 days), and Prolonged (more than 7 days). These categories reflect general estimates made by the physician authors and reviewers of the care guidelines based on their judgment, review of the literature, and database analyses. It is expected that the stay extension would be added to the GLOS but that multiple conditions would result in adding only the greatest of the extensions (ie, care for conditions would be simultaneous).

Care Planning

The Care Planning section contains information on common tests, procedures, treatments, consultations, and other care steps that may be needed during the patient stay in the defined level of care. It helps care planners and providers identify scheduling needs and other care issues (eg, test results to be obtained, treatments to be completed). The lists identify possible care needs, not necessarily what is expected during a recovery course. Only a small set of items listed may apply for any given patient. Also listed are key items to be monitored during the hospital stay as indicators of patient recovery, possible deterioration, and increased care needs.

Discharge Criteria

The Discharge Criteria section in non-inpatient guidelines contains the defining criteria for each of the Recovery Milestones in the last stage of the Recovery Course. The criteria identify patients who may be appropriate for discharge from the specific level of care due to

adequate patient stabilization or improvement. In addition, the criteria identify patients for whom the level of care is no longer indicated because a higher level of care is indicated or because of refusal of treatment.

Discharge

The Discharge section contains information to help clinicians accomplish a safe discharge and lists possible discharge destinations.

Discharge Planning

The Discharge Planning section summarizes the discharge planning needs that are most important to accomplishing a safe discharge and immediate postdischarge course of care. It covers tasks that may need to be accomplished during the patient's stay or at the time of discharge from the relevant level of care.

Discharge Destination

Discharge Destination describes "Usual" and "Alternate" destinations for patient discharge. "Usual" specifies the anticipated discharge destination if the patient follows optimal recovery. It identifies if achieving the guideline objective would most commonly be accomplished by discharging the patient to, for example, outpatient care, or, alternatively, to maintenance care. The "Alternate" destination section specifies other destinations that may be needed at discharge for a significant proportion of patients covered by the guideline.

Evidence Summary

The Care Guidelines for Behavioral Health contain Evidence Summaries. Each of the 16 diagnostic groups has its own Evidence Summary. The summary is included at the end of each guideline within a diagnostic category (ie, each level of care). These summaries are designed to present key evidence concerning the disorder in a concise manner. The Background section of the Evidence Summary contains information describing the disorder's features, diagnostic criteria, affected patient populations, and the like. The Treatment section describes, where available, evidence concerning pharmacologic and/or psychosocial treatments for the disorder. Where available, geriatric-specific and child/adolescent-specific evidence is also included. The Level of Care section includes information and description of evidence concerning what patient populations may require various levels of care (ie, inpatient, partial hospital). The evidence supporting the inpatient level of care Goal Length of Stay (eg, analysis of national all-payer databases) is presented in the inpatient guidelines. The Remission and Relapse section of the Evidence Summary contains information concerning the nature of the disorder over time, specifically chronicity, and description of factors and treatments that influence remission and relapse rates.

References

The care guidelines include a full reference list that identifies the sources used to develop specific guideline content. Links to references also appear as numbers within parentheses throughout guideline content. Note that a particular reference may pertain to some but not all of the statements made in a guideline section. Furthermore, references are cited for specific pieces of information contained in the article; some of the content within the article may not be pertinent to best practice. Clicking on the reference number (hyperlink) generates a pop-up box displaying the source. Users also can click on the references link at the top of the guideline or scroll down to the References section. The references list displays sources numerically in the order in which they appear in the text. For each reference, a link is provided to the abstract of the published article when it is available in the public domain, and a digital object identifier (DOI) is also included, when one exists.

Footnotes

Footnotes appear in the guidelines as letters within brackets. Footnotes provide the user with additional clinical and operational information pertinent to the section after which the footnote letter appears. Clicking on the footnote letter generates a pop-up box displaying the full text of the footnote. Users can also click on the Footnotes link at the top of the guideline or scroll down to the Footnotes section to review the text of all footnotes. The footnotes are listed in the order in which they appear in the text.

Other Behavioral Health Care Guidelines

Behavioral Health Care also delivers specific evidence-based guidelines related to the assessment, management, and treatment of behavioral health conditions. These guidelines are categorized as follows:

- Opioid Management
 - Medications
- Specialty Medications
- Community Services
- Testing Procedures
- Therapeutic Services

Clinical Indications

The Clinical Indications section contains the specific criteria that identify when an intervention, medication, service, diagnostic test, procedure, or treatment may be indicated. If none of the criteria apply, the appropriateness of using the resource may be questioned. It is important to note that every medical situation must be considered individually, and there may be reasons to utilize a resource even when

the criteria do not apply. The section uses a format of "ALL of the following" or "1 or more of the following," with nested lists of criteria. On occasion, "2 or more of the following" or "3 or more of the following" is used. Often, there are subsequent subset levels of criteria that may display within expandable and collapsible sections (the user should click the "+" to expand the content). The phrase "ALL of the following" requires that all of the subsequent child bullets be met; the phrase "1 or more of the following" requires that only one of the subsequent child bullets needs to be met. For some guidelines, authorization criteria are not provided, and the following phrase is used instead: "Current Role Remains Uncertain."

Current Role Remains Uncertain

Guidelines with the designation "Current Role Remains Uncertain" have no Clinical Indications. "Current Role Remains Uncertain" is a statement used when the available evidence regarding an intervention, medication, service, diagnostic test, procedure, or treatment is inadequate to support indications for its use. In some instances, this is because available evidence is insufficient to justify clinical indications. In other instances, existing evidence is conflicting or negative, which precludes listing clinical indications for the medical resource. The use of this statement does not mean that the intervention, medication, service, diagnostic test, procedure, or treatment lacks value or that, after careful consideration, it may not be useful for a specific patient in a specific clinical situation.

Alternative Care Planning - Other Behavioral Health Care Guidelines

The 2 Opioid Management guidelines include an Alternative Care Planning section. This section identifies levels of care or treatments that may be appropriate for some or many patients to use in lieu of the resource that is the subject of the guideline. The section is provided to assist with treatment planning, particularly when it has been determined that the requested medical resource is not appropriate or available for the patient at this time.

Care Treatment

The 2 Opioid Management guidelines include a Care Treatment section. The Care Planning and Evaluation section contains information on common tests, procedures, treatments, monitoring, consultations, and other care steps that may be needed for a patient. It helps care planners and providers identify scheduling needs and other care issues (eg, test results to be obtained, treatments to be completed). The lists identify possible care needs, not necessarily what is expected during a recovery course. Only a small set of items listed may apply for any given patient. Also listed are key items to be monitored as indicators of patient recovery, possible deterioration, and increased care needs. The Continuing Care section outlines factors that may be considered when deciding how long the course of treatment should be continued.

Evidence Summary - Other Behavioral Health Care Guidelines

The Evidence Summary provides a summary of the evidence used to develop the guideline. It provides a compilation of the information from all the annotations that appear in the guideline. The summary is not intended to be a substitute for the full-text articles. Interpretations of the medical literature may vary, and the reader's review of all relevant literature is encouraged.

The annotations in Behavioral Health Care are brief summaries of supporting evidence for Optimal Recovery Guidelines, Opioid Management, Specialty Medications, Community Services, Testing Procedures, and Therapeutic Services content. The annotations are located near the guideline sections to which they pertain. The annotations allow quick access to the evidence. Information from a given source (eg, article from the literature) may appear in more than one annotation in a given guideline, with each usage emphasizing different aspects or data from that source. An annotation is represented by an "N" icon; to access the content, users click on the icon, and a pop-up box displaying the annotation contents will appear.

There are 8 types of annotations in Behavioral Health Care, but not all 8 annotation types will appear in every guideline. Background annotations provide a definition of a diagnosis or procedure, if needed. Criteria annotations include information such as evidence regarding who should be admitted or get a procedure or service, what steps can reduce admission rates, what the appropriate indications and settings for a procedure are, what the advantages and disadvantages of various options to a procedure are, if such exist. Alternatives annotations include information such as evidence supporting use of alternatives to inpatient care or procedure. Length of Stay annotations present evidence that supports the Goal Length of Stay (GLOS). Treatment annotations provide a summary of the evidence that is available to support specific procedures, interventions, or therapies. Inconclusive or Non-Supportive Evidence annotations summarize the evidence for patients or situations for which the specific procedure, intervention, or therapy may not be appropriate. Level of Care annotations present evidence reflective of the multiple levels of care that are available for the management of people with behavioral health conditions. Finally, Remission and Relapse annotations present evidence related to treatments that may be associated with remission or relapse, and which can reduce the likelihood of readmission or improve outcomes.

Each statement within an annotation is supported by one or more citations, and this evidence base is evaluated by the physician editors and graded according to the following hierarchy:

- **(EG 1) Evidence Grade 1:**
 - Meta-analyses
 - Randomized controlled trials with meta-analysis
 - Randomized controlled trials
 - Systematic reviews
- **(EG 2) Evidence Grade 2:**

- Observational studies; examples include:
 - Cohort studies with statistical adjustment for potential confounders
 - Cohort studies without adjustment
 - Case series with historical or literature controls
 - Uncontrolled case series
- Published guidelines
- Statements in published articles or textbooks
- **(EG 3) Evidence Grade 3:**
 - Unpublished data; examples include:
 - Large database analyses
 - Written protocols or outcomes reports from large practices
 - Expert practitioner reports

In select Behavioral Health Care Guidelines (Medications, Specialty Medications, Testing Procedures, Therapeutic Services), Recommendation Grades provide additional insight into the reasoning underlying certain recommendations and the strength of the recommendation. One of three different Recommendation Grades may be assigned to Criteria annotations, as follows:

- **RG A1:** Evidence demonstrates at least moderate certainty of at least moderate net benefit.
- **RG A2:** Evidence demonstrates a net benefit, but of less than moderate certainty, and may consist of a consensus opinion of experts, case studies, and common standard care.
- **RG A3:** Evidence demonstrates an incomplete assessment of net benefit vs harm; the drug is currently approved by a federal regulatory agency.

Each area of uncertainty that is discussed under the heading "Inconclusive or Non-Supportive Evidence" has been assigned a Recommendation Grade summarizing the evidence base for that indication. One of three different Recommendation Grades is used for each area of uncertainty, as follows:

- **RG B:** Evidence is insufficient, conflicting, or poor and demonstrates an incomplete assessment of net benefit vs harm; additional research is recommended.
- **RG C1:** Evidence demonstrates a lack of net benefit; additional research is recommended.
- **RG C2:** Evidence demonstrates potential harm that outweighs benefit; additional research is recommended.

References

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Footnotes

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Definitions

Definitions of medical terms and healthcare concepts are used extensively throughout guideline content. Definitions are included to present standardized meanings or values whenever possible. Definitions appear as blue text with active hyperlinks. Clicking a Definition link generates a pop-up box displaying the full text of the definition.

Behavioral Health Scoring Tools

Behavioral Health Care also provides scoring tools related to the assessment of behavioral health conditions. There are adult and child or adolescent versions of these tools. These tools were created based on multiple professional society guidelines.

The scoring tools require the assessment of the patient across multiple behavioral health dimensions and can be used to evaluate the patient for admission, concurrent stay, and discharge.

The scoring tools include a full reference list that identifies the sources used to develop the content. Links to references also appear as numbers within parentheses throughout the scoring tool. Clicking on the reference number (hyperlink) generates a pop-up box displaying

the source. Users also can click on the references link at the top of the guideline or scroll down to the References section. The references list displays sources numerically in the order in which they appear in the text. For each reference, a link is provided to the abstract of the published article when it is available in the public domain, and a digital object identifier (DOI) is also included, when one exists.

Use of the Behavioral Health Scoring Tools is at the user's discretion. An example of a potential use case for the Behavioral Health Scoring Tools is the ability to review a patient's care journey using the same criteria. Specific regulatory requirements may lead the user to choose this tool.

As always, the Behavioral Health Scoring Tools are designed to assist the clinical review process that is needed to render judgment about cases in which admission, concurrent stay, or discharge may be appropriate. The scoring tool recommendations are not inflexible expectations for all patients but should be considered as guides to quality care. The Behavioral Health Scoring Tools are not designed to be used in isolation but rather in conjunction with clinical judgment.

Home Care and Recovery Facility Behavioral Health Guidelines

Behavioral Health Care includes guidelines to help manage behavioral health patients in the recovery facility and home settings. Recovery Facility Care guidelines contain Clinical Indications for Admission to Recovery Facility, Alternative Levels of Care, a General Treatment Course, and a Discharge Planning section. Home Care guidelines contain Clinical Indications for Admission to Home Healthcare, a Home Healthcare Treatment Plan, and a Discharge Planning section.

Home Care Visit Data

Visit tables are designed to allow organizations to define their goals for home healthcare utilization by choosing based on payer type (Commercial or Medicare) within the displayed range. Individual patients may require fewer or more visits as appropriate for their clinical status and care needs, and organizations may choose to authorize based on a certain percentile display based on organizational policy and procedure.

Home care use for condition refers to the frequency with which home care services are provided after inpatient hospitalization for this diagnosis or procedure.

Registered nurse visits are displayed as percentiles that are based on the observed utilization of registered nurse visits for the diagnosis.

Home health aide visits are displayed as percentiles that are based on the observed utilization of home health aide visits for the diagnosis.

Rehabilitation therapy visits are displayed as percentiles that are based on the observed utilization of therapy visits for the diagnosis.

Home Care Visit Data Methodology

The home healthcare visit goals presented in this product are divided into 2 categories: Commercial and Medicare. The commercial data are based on a national commercial insurance database of patients with home health claims during 2021 and 2022. The Medicare data are based on the Medicare database of patients with home health claims during 2021. The commercial visit data displays the number of visits that should occur within that episode (which is identified on the basis of continuous periods of home care service with a gap of no more than 30 days). The Medicare visit data displays the number of visits that should occur within that episode (which is identified on the basis of a 30-day payment period).

The visit data are displayed as percentiles that are based on the observed utilization of all discipline visits for the particular diagnosis. Organizations may choose to use either the 10%ile to 20%ile or the 20%ile to 40%ile data for skilled visits, or they may choose another set of percentiles to determine optimal visit goals.

Where the volume of data is insufficient, clinically similar guidelines are combined for purposes of analysis. This assignment is based on the nature of their prior acute care hospitalizations. This categorization is consistent with the structure of the Home Care Behavioral Health guidelines. However, to the extent that home healthcare may sometimes be needed for conditions other than the principal reason for the prior hospitalization, this approach tends to introduce greater heterogeneity into some patient groups.

Also available is information on the frequency of home care use for a particular condition. This **Home care use for condition** information identifies the percentage of inpatient admissions followed by home care after an acute episode. **Home care use for condition** refers to the frequency with which home care services are provided after inpatient hospitalization for a diagnosis or procedure: Rare: less than 5%; Occasional: from 5% up to 20%; Frequent: from 20% up to 40%; Very Frequent: 40% or greater.

Each guideline lists the professionals who frequently perform visits, along with applicable visit goals, with the assumption that the services of each provider listed will vary from case to case. Not all patients categorized in a given guideline require all the types of home healthcare that are presented.

The visit goals are reviewed by practitioners to ensure that they can be achieved successfully. However, clinical judgment is always needed to determine the number and type of visits required for a particular patient.

Recovery Facility Care Length of Stay Methodology

The lengths of stay for Recovery Facility Care (Medicare) are based on observed lengths of stay for a national sample of Medicare patients admitted during 2021. The lengths of stay for Recovery Facility Care (Commercial) are based on observed lengths of stay for a large payer database sample of patients admitted during the period from 2021 to 2022. Patients are assigned to the Optimal Recovery Guideline categories based on the nature of their prior acute care hospitalization. Such categorization is consistent with the structure of the Recovery Facility Behavioral Health guidelines. However, to the extent that recovery facility care may sometimes relate to conditions other than the condition that was the principal reason for the prior hospitalization, this approach will tend to introduce greater heterogeneity into the patient groups identified in this fashion.

The Length of Stay table is designed to represent the lengths of stay attained by a substantial proportion of patients. The length of stay is presented in a range of 10%ile to the 50%ile of the observed distribution of length of stay for the guideline in the national Commercial and Medicare samples. This is designed to allow organizations to define their goals for recovery facility utilization by choosing from the displayed range. A dash ("-") in the table indicates that there is insufficient data available for that diagnosis.

The recovery facility lengths of stay are empirically derived from observed treatment patterns rather than being based primarily on the published medical literature. Because of the frequency and variety of complications of care, comorbidities, functional limitations, and other patient-specific issues in the recovery facility population, the recovery facility lengths of stay require more modification based on professional judgment than the inpatient hospitalization goals. Case managers should expect that each patient in a recovery facility will have a written plan of care, including length of stay goals, and that progress will be documented in notes from each functional area. It is realistic to receive weekly, rather than daily, updates of progress toward goals. Patients may be discharged from the facility or transferred to a lower level of care within the facility as needs decrease, either through attainment of goals or through recognition of achievement of maintenance level.

The 50%ile length of stay does not represent average length of stay. It can be used as a reference point to compare local patient management against the national database. The data used include patients with complications or comorbidities who were admitted for a specific primary diagnosis.

Recovery Facility Behavioral Health lengths of stay are shown as a percentile data table:

Commercial Length of Stay					Medicare Length of Stay				
10%ile	20%ile	30%ile	40%ile	50%ile	10%ile	20%ile	30%ile	40%ile	50%ile
Days	Days	Days	Days	Days	Days	Days	Days	Days	Days

Abbreviations

The Abbreviations resource lists common medical and care guidelines abbreviations and their corresponding definitions. This resource also provides definitions for units of measurement.

Care Management Tools

Care Management Tools provide additional information on managing particular elements of care. These tools aid case managers and other healthcare professionals in creating and managing the treatment plan, and they help educate the patient and caregiver about treatment. Included in this section are various tools and calculators that address patient monitoring and assessment, and management of specialized types of care. The Care Management Tools section also provides definitions of levels of care across the continuum.

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